DERMALUXE

BY MEDICARE · SKIN & HAIR CLINICS

ఏలూరు · Eluru

STUDY MATERIAL · TRAINER MANUAL

Hair Care Treatments

హెయిర్ కేర్ ట్రీట్‌మెంట్స్ — 30 రోజుల స్టడీ మెటీరియల్

30 training days · Sundays holiday · daily theory, hands-on, safety, key points, homework and a professional tip. Trainer: Dr. Meghana Valeti, MD DVL.

DAY PLAN · రోజువారీ ప్రణాళిక

01 Welcome, clinic protocol & professional grooming

02 Hair anatomy & the hair growth cycle

03 Types of hair loss — pattern, diffuse, patchy

04 Scalp conditions — dandruff, seborrhoea, psoriasis, fungal

05 Scalp & hair analysis — trichoscopy and documentation

06 Consultation, consent, nutrition & lifestyle counselling

07 Scalp cleansing & detox protocols

08 Anti-dandruff treatment protocol

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17 Hair mesotherapy — cocktails & technique

18 Microneedling, derma roller & LLLT for scalp

19 Combination protocols & treatment course design

20 Complications, infection control & emergencies

21 Case studies & treatment planning workshop

22 Client communication, expectations & retention

23 Clinic operations, WhatsApp flow & documentation

24 Revision + full live-client day

25 Mock practical, viva prep & portfolio

26 Advanced session: combination clinics & upselling ethically

27 Hair transplant awareness & referral (observation only)

28 Records, photography review & progress reporting

29 Final revision & exam readiness

30 Final practical exam, viva & career guidance

Day 01

1 of 30 · Sundays holiday

Welcome, clinic protocol & professional grooming

స్వాగతం, క్లినిక్ ప్రోటోకాల్ & ప్రొఫెషనల్ గ్రూమింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand how a trichology clinic runs and your role in it
- Learn hygiene, sterilisation and biomedical waste rules
- Present yourself as a clinic professional

THEORY · థియరీ

The hair patient journey

Enquiry → consultation with the dermatologist → scalp analysis → diagnosis and plan → procedures (PRP/GFC/meso/scalp treatments) → home care → monthly review with photographs. Hair patients are anxious and have usually tried five things before you. Your calm explanation of WHY previous things failed is what converts them.

Hygiene & biomedical waste

Hair procedures involve blood (PRP) and needles — so protocol is stricter than skin. Hand hygiene, gloves for every client, surface disinfection with 70% IPA, autoclave for metal instruments. Biomedical waste segregation: red bag (contaminated plastic — syringes without needles, tubes), white/blue puncture-proof container (needles, sharps), yellow (blood-soaked gauze). Never recap a needle.

Professional grooming

Your own hair and scalp are your advertisement. Clean tied hair, clean nails, pressed uniform, closed shoes, no strong perfume. Learn to speak about hair loss without ever making a client feel ashamed — no 'tala meeda emi ledu' jokes, ever.

Hands-on today

- Clinic tour: consultation, procedure room, centrifuge area, sterilisation, waste zone
- Practise 6-step handwash, glove technique, waste segregation with dummy items
- Set up a PRP trolley and have it audited

Safety & contraindications

- Sharps go straight into the puncture-proof container
- Any needle-stick injury: wash, do not squeeze, report immediately, follow protocol

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Red / yellow / sharps segregation
- Autoclave metal instruments
- Never recap needles
- Hair clients are anxious — be gentle

Homework

Write the biomedical waste chart from memory and stick it above your trolley.

Grooming & professional tip

Never laugh at, or joke about, hair loss — not even with colleagues. One overheard comment loses a client forever.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Hair anatomy & the hair growth cycle

హెయిర్ నిర్మాణం & గ్రోత్ సైకిల్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Describe the hair follicle structure and each part's function
- Explain anagen, catagen and telogen simply to a client
- Link the cycle to why treatments take months

THEORY · థియరీ

Follicle structure

Bulb (matrix cells that divide fastest in the body), dermal papilla (blood supply and growth signalling — the real target of PRP and GFC), bulge (stem cells), sebaceous gland, arrector pili muscle, and the shaft with cuticle, cortex and medulla. Damage to the papilla area = miniaturisation; damage to the bulge = permanent loss.

The cycle

Anagen (growing) 2–7 years — normally 85–90% of hairs; catagen (transition) 2–3 weeks; telogen (resting/shedding) 3 months — normally 10–15%. Losing 50–100 hairs a day is normal. In telogen effluvium the telogen share jumps to 20–50% and shedding becomes dramatic 2–3 months AFTER the trigger (fever, surgery, delivery, crash diet, COVID, severe stress).

Why results take months

A treatment started today can only act on follicles entering anagen. Visible new growth takes 3 months; thickness improvement 4– 6 months; full assessment at 9–12 months. Every hair client must be told this in the first consultation, or they will quit at week 6 and blame the clinic.

Hands-on today

- Label a follicle diagram from memory
- Explain the hair cycle to a partner in Telugu in 90 seconds
- Do a pull test on 3 volunteers and record the result

Safety & contraindications

- Sudden patchy loss, scaling, pain or scarring → doctor review the same day

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 85–90% anagen is normal
- 50–100 hairs/day is normal
- TE shows 2–3 months after the trigger
- New growth visible at 3 months

Homework

Draw the hair cycle with timelines and write the 3-month rule in your own words.

Grooming & professional tip

Draw the cycle on paper for the client. People who can see why it takes 3 months stop panicking and stay in treatment.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 03

3 of 30 · Sundays holiday

Types of hair loss — pattern, diffuse, patchy

హెయిర్ లాస్ రకాలు

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Recognise the main hair-loss patterns
- Know which are therapist-treatable and which are strictly medical
- Ask the right history questions

THEORY · థియరీ

Androgenetic alopecia

Male pattern: temporal recession and vertex thinning — Norwood I–VII. Female pattern: widening central parting with a preserved frontal line — Ludwig I–III. Driven by DHT sensitivity and genetics; progressive without treatment. Responds well to combined medical + procedural care (PRP/GFC/meso) started early.

Telogen effluvium and nutritional loss

Diffuse shedding all over, 2–3 months after a trigger; positive pull test; usually self-limiting in 6 months once the trigger is fixed. Check for anaemia, thyroid disease, vitamin D and B12 deficiency, PCOS, crash dieting, post-partum and post-fever states — all doctor-ordered investigations.

Patchy and scarring loss

Alopecia areata: sudden smooth round patches, exclamation-mark hairs — autoimmune, medical treatment. Tinea capitis: scaling, broken hair, itching, often children — fungal, medical. Scarring alopecias (lichen planopilaris, frontal fibrosing): loss of follicular openings, redness, scaling — urgent doctor referral, no procedures. Traction alopecia: from tight hairstyles, reversible if caught early.

Hands-on today

- Classify 8 clinical photos by pattern (Norwood/Ludwig where relevant)
- Take a full hair history from a volunteer using the clinic form
- Identify red-flag findings from a list of 10 case snippets

Safety & contraindications

- No PRP, GFC or meso on undiagnosed patchy loss, scaling scalp or scarring alopecia
- Children with patchy loss → always doctor first

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Norwood (M) / Ludwig (F)
- TE: diffuse, trigger 2–3 months back
- Loss of follicular openings = scarring, refer
- Traction loss is preventable

Homework

Make a one-page red-flag list that means 'stop and call the doctor'.

Grooming & professional tip

Ask every woman about her last delivery, thyroid report and diet. Half of female diffuse loss in our clinic is nutritional or hormonal, not genetic.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 04

4 of 30 · Sundays holiday

Scalp conditions — dandruff, seborrhoea, psoriasis, fungal

స్కాల్ప్ కండిషన్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Differentiate the common scalp conditions on sight
- Plan a scalp treatment that supports the doctor's medical plan
- Counsel washing frequency and home care correctly

THEORY · థియరీ

Dandruff & seborrhoeic dermatitis

Malassezia yeast + oil + individual sensitivity → white/yellow greasy flakes, itching, redness. Seborrhoeic dermatitis is the inflamed end of the same spectrum and may also affect eyebrows, nose folds and chest. Chronic and relapsing: control, not cure. Flares with stress, cold weather, oily hair products and infrequent washing.

Psoriasis vs fungal

Scalp psoriasis: thick silvery well-defined plaques, may extend past the hairline, often with nail pitting — medical. Tinea capitis: scaling with broken hairs, black dots, sometimes boggy swelling (kerion), itchy, contagious, common in children — medical, antifungal by mouth. Never treat either with cosmetic scalp therapy alone.

Washing and home care

The biggest myth in our region: 'washing causes hair fall'. Oily, flaky scalps need washing every 1–2 days with the right shampoo. Medicated shampoos (ketoconazole, zinc pyrithione, selenium sulphide, salicylic) must be left on the scalp for 3–5 minutes. Heavy overnight oiling worsens dandruff — advise oil for 1–2 hours before wash, if at all.

Hands-on today

- Examine 4 volunteers' scalps with magnification and classify findings
- Demonstrate correct medicated-shampoo application and contact time
- Write a home-care plan for an oily, dandruff-prone client

Safety & contraindications

- Contagious scalp infection → no shared towels or tools; sterilise everything
- Do not perform scalp peels or PRP over active infection

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Malassezia + oil + sensitivity
- Medicated shampoo contact 3–5 min
- Wash more, not less, for dandruff
- Psoriasis & tinea = medical

Homework

Write the 'washing does not cause hair fall' explanation in Telugu, 6 lines.

Grooming & professional tip

Correcting the washing myth in the first consultation immediately marks you as the professional who actually knows hair.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 05
5 of 30 · Sundays holiday

Scalp & hair analysis — trichoscopy and documentation

స్కాల్ప్ & హెయిర్ అనాలిసిస్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Perform a complete scalp examination and trichoscopy
- Document density, miniaturisation and scalp condition
- Take standard photographs a doctor can compare

THEORY · థియరీ

Examination sequence

History → look at the parting width, hairline, vertex and temples → pull test (grasp 50–60 hairs, gentle traction; more than 6 hairs is positive) → look at the scalp itself (scaling, redness, pustules, follicular openings) → trichoscopy → photographs. Always compare left and right, and affected vs unaffected.

Trichoscopy basics

Look for: hair diameter diversity (>20% = miniaturisation, classic AGA), yellow dots (empty follicles), black dots and exclamation marks (alopecia areata), perifollicular scaling and erythema (inflammation), and loss of follicular openings (scarring — refer). Save images with the client's file.

Standard photographs

Fixed distance, same light, same angles every visit: frontal hairline, mid-scalp parting, vertex from above, both temples. Use a parting comb and the same background. These photos are the only honest measure of progress in 3 months — and they are the single best sales tool for continuing treatment.

Hands-on today

- Full scalp analysis with trichoscopy on 2 volunteers with written findings
- Photograph all 5 standard views and file them
- Perform and interpret a pull test

Safety & contraindications

- Never call a diagnosis — describe findings and route to the doctor
- Disinfect the dermoscope lens between clients

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Pull test >6 hairs = positive
- Diameter diversity >20% = AGA
- Loss of openings = scarring, refer
- Same 5 photo views every visit

Homework

Create your own scalp analysis card template and fill it for one family member.

Grooming & professional tip

Take the Day-0 photos before you say anything about results. Without them you have nothing to show at month three.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 06

6 of 30 · Sundays holiday

Consultation, consent, nutrition & lifestyle counselling

కన్సల్టేషన్, కన్సెంట్ & న్యూట్రిషన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Run a full hair consultation that uncovers the real cause
- Take proper consent for blood-based procedures
- Give practical nutrition and lifestyle guidance without prescribing

THEORY · థియరీ

Consultation structure

Onset and duration → pattern (diffuse/patchy/pattern) → shedding vs thinning → triggers (illness, delivery, surgery, diet, stress, COVID) → family history → medical history (thyroid, anaemia, PCOS, autoimmune) → medications → previous treatments and why they stopped → hair practices (oiling, colouring, straightening, tight styles) → expectations. Write it all down.

Consent for PRP/GFC

Blood-based procedures need specific informed consent: procedure, that their own blood is drawn and processed, expected 3–6 sessions, realistic outcome (reduced shedding first, then density), possible side effects (pain, swelling, headache, bruising, rare infection), and the need for maintenance. Signed, dated, with photo consent separate.

Nutrition & lifestyle

Practical, non-prescription advice: adequate protein at every meal (dal, eggs, paneer, chicken, curd), iron-rich foods with vitamin C, no crash diets, 7–8 hours of sleep, stress management, scalp-friendly hairstyles (no tight ponytails or braids), heat protection, and no daily chemical straightening. Deficiencies are confirmed and corrected by the doctor with reports — never sell supplements on your own.

Hands-on today

- Take 2 full hair histories using the clinic form and present them
- Complete a PRP consent form and check it for gaps
- Build a 7-day protein-focused diet suggestion sheet

Safety & contraindications

- Never advise stopping or starting any prescribed medicine
- Refer for blood tests through the doctor, not on your own initiative

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- History finds the cause more often than the machine
- Separate consent for blood procedures
- Protein + iron + sleep
- No supplement selling by therapists

Homework

Write a hair-consultation script in Telugu covering all 10 history points.

Grooming & professional tip

The client who feels fully questioned trusts the plan. Ten minutes of good history beats ten minutes of machine talk.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 07
7 of 30 · Sundays holiday

Scalp cleansing & detox protocols
స్కాల్ప్ క్లెన్సింగ్ & డీటాక్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Perform a professional scalp cleansing and detox treatment
- Select products by scalp type
- Handle the wash station and client comfort correctly

THEORY · థియరీ

Why detox

Product build-up, hard water minerals, oil, sweat and pollution block follicular openings and worsen dandruff and itching. A scalp detox clears this, improves the penetration of later treatments (serums, PRP, meso) and gives immediate relief from itch — which is why it is often the first session in any hair package.

Protocol

Analyse → section the scalp in 4 quadrants → apply detox/clarifying solution with a nozzle applicator along partings → gentle massage 5–8 minutes, fingertips only, never nails → optional steam or warm towel 5 minutes → thorough wash with the correct shampoo, two lathers → conditioner on lengths only, never on the scalp for oily types → towel dry → serum → dry with cool air.

Products by scalp type

Oily/dandruff: salicylic or ketoconazole-based clarifying. Dry/sensitive: mild, sulphate-free, soothing (aloe, panthenol). Post-chemical (colour/straightening): protein + moisture balance. Always do this AFTER analysis, never before — cleansing removes the evidence you need to see.

Hands-on today

- Full scalp detox on a live client, 45 minutes
- Practise sectioning and nozzle application on a mannequin
- Wash-station drill: client positioning, water temperature, eye and ear protection

Safety & contraindications

- Water temperature lukewarm, never hot — hot water worsens shedding and irritation
- Never use nails on the scalp; never scrub an inflamed scalp

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Analyse before you wash
- 4-quadrant sectioning
- Fingertips, not nails
- Conditioner on lengths only

Homework

Write your own 10-step scalp detox protocol card.

Grooming & professional tip

A great scalp massage is the most re-booked service in any hair clinic. Practise pressure and rhythm until it is genuinely relaxing.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 08

8 of 30 · Sundays holiday

Anti-dandruff treatment protocol

యాంటీ-డాండ్రఫ్ ట్రీట్‌మెంట్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Deliver a complete clinic anti-dandruff protocol
- Build a 6-week home plan that prevents relapse
- Know when dandruff is actually something else

THEORY · థియరీ

Clinic protocol

Analysis → gentle de-scaling (salicylic or urea-based lotion, 10–15 minutes under steam/cap) → medicated shampoo wash with 3–5 minute contact → anti-fungal/anti-inflammatory scalp serum → light massage → cool dry. Weekly for 4 weeks, then every 2– 3 weeks for maintenance.

Home plan

Medicated shampoo 2–3 times a week (rotate actives to avoid resistance), normal mild shampoo in between, no heavy overnight oiling, wash within 1–2 hours if oil is applied, wash after workouts, clean pillow covers and combs weekly, and no sharing of combs or towels. Warn that flakes will return if they stop — this is control, not cure.

When it is not dandruff

Thick silvery plaques (psoriasis), broken hairs with black dots (tinea), pustules and pain (folliculitis), or scaling with hair loss and lost follicular openings (scarring alopecia) are all medical. Cosmetic treatment alone will fail and delay real care — route them to the dermatologist.

Hands-on today

- Perform a full anti-dandruff treatment on a live client
- Write a 6-week home plan with shampoo rotation
- Examine 3 scalps and decide: cosmetic or refer

Safety & contraindications

- Do not de-scale over open, bleeding or inflamed skin
- Stop and refer if there are pustules, pain or spreading redness

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Contact time 3–5 minutes
- Rotate actives
- Control, not cure
- Clean combs and pillow covers

Homework

Design a shampoo rotation calendar for one month for a severe dandruff client.

Grooming & professional tip

Give dandruff clients a written 6-week calendar. Relapse usually happens because they stop the moment flakes disappear.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 09

9 of 30 · Sundays holiday

Scalp peels, exfoliation & hair spa treatments

స్కాల్ప్ పీల్స్ & హెయిర్ స్పా

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Perform scalp exfoliation and hair spa treatments safely
- Choose between treatments based on scalp and hair condition
- Add measurable value with a structured spa protocol

THEORY · థియరీ

Scalp peels

Low-strength salicylic, lactic or glycolic solutions applied along partings loosen keratin build-up and clear follicular openings. 3–7 minutes, mild tingling, neutralise/wash as per brand. Excellent before a PRP or meso series. Contraindicated on inflamed, infected or freshly coloured scalp (wait 7 days).

Hair spa

Cleanse → mask/ampoule as per hair condition (protein for chemically damaged, moisture for dry frizzy, balancing for oily roots) → steam 10 minutes → scalp and shoulder massage → wash → serum → blow dry cool. 60 minutes, monthly. This is your retention service: relaxing, visible shine, and an easy monthly booking.

Sequencing with medical work

Scalp peel and PRP: peel at least 3 days before, never same day. Hair spa after PRP: wait 48–72 hours. Colour/chemical services: 7 days away from any clinical scalp procedure. Write the sequence into the client's calendar.

Hands-on today

- Perform one scalp peel and one full hair spa on live clients
- Select masks correctly for 4 different hair conditions
- Draw a 4-week calendar combining spa, peel and PRP

Safety & contraindications

- No scalp peel within 7 days of colouring, straightening or smoothing
- Protect the face, ears and neck; never let peel run into the eyes

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Peel 3 days before PRP
- Spa 48–72 h after PRP
- Protein vs moisture masks
- Monthly spa = retention

Homework

Write mask-selection notes for 5 hair types you see most in Eluru.

Grooming & professional tip

Hair spa is where you learn a client's real routine — they talk during the massage. Note what they say; it changes the plan.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Hair fall protocols — men & women

హెయిర్ ఫాల్ ప్రోటోకాల్స్ — పురుషులు & మహిళలు

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Build a staged hair-fall plan alongside the doctor's medical plan
- Adapt the approach for male pattern vs female diffuse loss
- Set honest milestone expectations

THEORY · థియరీ

Staged plan

Stage 1 (0–4 weeks): investigations by the doctor, scalp health correction (dandruff/seborrhoea), nutrition and hair-practice correction, Day-0 photos. Stage 2 (1–4 months): procedures — PRP or GFC monthly ×4, meso in between if advised, plus the doctor's topical/oral plan. Stage 3 (4–12 months): maintenance every 2–3 months, 3-monthly photo reviews.

Male pattern specifics

Norwood grading, earlier intervention gives far better results, vertex responds better than the frontal hairline, and realistic outcome is arresting loss plus 15–30% density gain — not a new hairline. Clients wanting a hairline restored need a hair transplant consultation, which is a separate surgical service.

Female specifics

Always rule out anaemia, thyroid, PCOS, post-partum and crash dieting with the doctor first. Women often have both diffuse shedding and pattern thinning together. Counsel on traction: tight ponytails, braids, extensions and daily heat. Results measured at the parting width and vertex photos.

Hands-on today

- Build full plans for one male Norwood III and one female Ludwig II case
- Present the milestone timeline to a partner as if to a client
- Practise the 'no new hairline' conversation

Safety & contraindications

- Do not start procedures before the doctor's investigations are back
- Sudden severe shedding with scalp symptoms → doctor, not procedures

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Photos at Day 0, month 3, 6, 12
- Arrest first, density second
- Early treatment = better results
- Transplant is a separate conversation

Homework

Write a 12-month treatment calendar for a Norwood III client.

Grooming & professional tip

Always state the goal as 'shedding taggadam mundu, density tarvata'. Clients who understand the order stay through month two when nothing looks different yet.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 11

11 of 30 · Sundays holiday

Blood & platelets — the science behind PRP

బ్లడ్ & ప్లేట్‌లెట్స్ — PRP సైన్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Explain what PRP is and how it stimulates hair growth
- Understand blood components and what the centrifuge separates
- Prepare the theory you need before touching a tube

THEORY · థియరీ

Blood components

Whole blood = plasma (55%), buffy coat (platelets + white cells) and red cells (45%). Spinning separates them by density. Platelet-rich plasma is plasma with platelets concentrated 3–6× above baseline. Platelets contain alpha granules full of growth factors — PDGF, VEGF, EGF, IGF-1, TGF-β.

How it works on hair

Injected around the follicles, the growth factors increase dermal papilla cell activity and blood supply, prolong anagen, and reduce miniaturisation. Effect: less shedding by 4–6 weeks, visible new growth by 3 months, density improvement by 5–6 months. It supports — it does not replace — the doctor's medical therapy, and it cannot regrow follicles that are already scarred.

Tubes and activation

PRP tubes contain an anticoagulant (ACD-A or sodium citrate) and often a separating gel. Handle gently — rough shaking damages platelets. Some protocols activate with calcium chloride; many hair protocols use non-activated PRP. Always follow the doctor's chosen protocol exactly, including spin speed and time.

Hands-on today

- Identify tube types, anticoagulants and their uses
- Watch a full spin and identify the layers in the tube
- Write the clinic's exact PRP protocol from observation

Safety & contraindications

- Never use a tube past its expiry or with a broken seal
- Label every tube with the client's name immediately after draw — no exceptions

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Platelets concentrated 3–6×
- Growth factors: PDGF, VEGF, IGF-1
- Shedding reduces by 4–6 weeks
- Follow the clinic's spin protocol exactly

Homework

Draw a labelled tube after centrifugation and write which layer is used.

Grooming & professional tip

One mislabelled tube can mean someone else's blood in a client. Label at the chair, in front of the client, every single time.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 12
12 of 30 · Sundays holiday

Phlebotomy assist & sample handling
బ్లడ్ డ్రా అసిస్ట్ & శాంపిల్ హ్యాండ్లింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Assist a venepuncture safely and confidently
- Handle, label and transport samples correctly
- Manage a client who is anxious or faints

THEORY · థియరీ

Preparation

Confirm identity and consent, check the client has eaten and hydrated, seat them comfortably with the arm supported, apply the tourniquet 4 finger-widths above the elbow for no more than one minute, select the median cubital vein, clean with 70% alcohol in one outward spiral and let it dry fully.

During and after

The doctor or trained phlebotomist draws; you support the client, hold the tray, hand over tubes in the correct order, and invert gently 5–8 times immediately (never shake). After: release tourniquet before needle withdrawal, firm pressure with dry gauze 2–3 minutes, no bending of the elbow, tape, and observe for 5 minutes.

Vasovagal episodes

Pale, sweating, nausea, ringing in ears, yawning — act at the first sign: stop, lay the client flat, raise the legs, loosen tight clothing, cool cloth, fluids when alert, never leave them alone, call the doctor. Most faints happen in young, anxious, fasting clients — prevention is food, water and conversation.

Hands-on today

- Assist 2 supervised draws: set-up, tube order, inversion, pressure, disposal
- Practise tourniquet application and vein identification on colleagues (no needles)
- Simulate and manage a vasovagal episode

Safety & contraindications

- Never attempt a venepuncture yourself unless certified and permitted
- Needle-stick injury: wash with soap and water, do not squeeze, report immediately

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Tourniquet ≤ 1 minute
- Invert 5–8 times, never shake
- Pressure 2–3 minutes, no bending
- Faint: flat + legs up + call doctor

Homework

Write the step-by-step draw assist checklist and the faint protocol.

Grooming & professional tip

Keep talking to the client through the draw and make sure they never watch the needle. Distraction prevents more faints than anything else.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 13
13 of 30 · Sundays holiday

Centrifuge protocol & PRP preparation
సెంట్రిఫ్యూజ్ & PRP ప్రిపరేషన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Operate the centrifuge correctly and safely
- Prepare PRP to the clinic's exact protocol
- Maintain sterility from tube to syringe

THEORY · థియరీ

Centrifuge basics

Always balance: tubes of equal weight opposite each other; add a water-filled balance tube if you have an odd number. Close the lid, set speed and time exactly as protocol, never open before it stops completely. Typical single-spin hair PRP: around 1500 rpm for 8–10 minutes, or the brand's stated protocol — never improvise.

Separating the PRP

After the spin: plasma on top, buffy coat line, red cells at the bottom. Using a sterile syringe and long needle or the kit's transfer device, draw the plasma just above the buffy coat, including the buffy layer as per protocol, without disturbing the red cells. Aim for the volume the doctor asked for; typical scalp session 4–6 ml.

Sterility chain

Sanitise and glove, work on a sterile draped tray, open kits without touching inner surfaces, never leave a prepared syringe uncovered, use it within the protocol time window, and label with the client's name. Discard immediately if there is any doubt about contamination — a scalp infection is far more expensive than a kit.

Hands-on today

- Balance and run the centrifuge 3 times with dummy tubes
- Prepare PRP under supervision from a real draw and have the volume checked
- Set up and break down a sterile tray, audited

Safety & contraindications

- Never open a running centrifuge; never run unbalanced
- One client = one kit; never share or reuse anything

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Balance every spin
- Follow brand rpm and time exactly
- Typical scalp dose 4–6 ml
- Label and use within the time window

Homework

Write the centrifuge SOP for your clinic including balancing and cleaning.

Grooming & professional tip

Photograph your prepared syringe next to the labelled tube for the record. It takes five seconds and settles any future question.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 14

14 of 30 · Sundays holiday

Scalp anaesthesia, injection technique & assisting

స్కాల్ప్ నంబింగ్ & ఇంజెక్షన్ టెక్నిక్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Prepare the scalp and manage comfort for injections
- Understand the injection pattern, depth and dose
- Assist the doctor smoothly through the session

THEORY · థియరీ

Comfort options

Topical numbing cream (lidocaine-prilocaine) applied 30–45 minutes before under occlusion on the marked area, removed and cleaned fully before injection; ice or cold air during; vibration device as distraction; and, for sensitive clients, a doctor-administered ring block. Explain the sensation honestly — pressure and brief pricks, not sharp cutting.

Pattern, depth and dose

Injections in a grid roughly 1 cm apart across the thinning zone, depth in the dermis (about 4–6 mm) at a 45–90° angle, small aliquots (0.05–0.1 ml) per point, covering the whole affected area plus a 1 cm margin. A typical session uses 4–6 ml over 60– 100 points. The doctor injects; you mark, hand over, count and document.

Your role during the session

Client positioning, hair sectioning and clipping, antiseptic prep, marking the grid, handing syringes, gauze for spot bleeding, counting points, watching the client's face and vitals, and keeping the tray sterile. Speak to the client through it — a narrated session feels half as long.

Hands-on today

- Apply and time numbing cream correctly on a volunteer
- Section, mark and prep a scalp grid ready for injection
- Assist one full PRP session end to end and log it

Safety & contraindications

- Never exceed the recommended area or amount of numbing cream — lidocaine toxicity is real
- Stop and call the doctor for severe pain, dizziness or palpitations

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Numbing 30–45 min under occlusion
- Grid 1 cm apart, 4–6 mm deep
- 0.05–0.1 ml per point
- 4–6 ml per scalp session

Homework

Draw the injection grid for a Ludwig II female pattern case and count the points.

Grooming & professional tip

Count the points out loud with the doctor. It keeps the pattern even and shows the client the session is being done thoroughly.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 15

15 of 30 · Sundays holiday

PRP practical on live clients

PRP ప్రాక్టికల్ — లైవ్ క్లయింట్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Run a complete PRP session as the assisting therapist
- Manage the room, the client and the record without prompting
- Handle common reactions confidently

THEORY · థియరీ

Session flow

Confirm identity, consent and photos → history check (any illness, new medication, recent alcohol) → set-up and sterile tray → draw and spin → numbing → prep and marking → injection with the doctor → post-procedure care → aftercare counselling → record and rebook. Total 45–60 minutes.

Immediate reactions

Expected: pinpoint bleeding, mild swelling, tightness, mild headache for a few hours, tenderness for 1–2 days. Not expected: severe pain, spreading redness, pus, fever — these are reported to the doctor. Manage the expected ones with cold compress (not ice directly), paracetamol only if the doctor advises, and reassurance.

Aftercare

No head wash for 24 hours, no oil, no colouring for a week, no gym/sauna/swimming for 48 hours, no alcohol that day, sleep with a clean pillow cover, avoid direct sun. Normal shampoo after 24 hours. Next session in 4 weeks. Send the aftercare on WhatsApp the same evening and call at 24 hours.

Hands-on today

- Assist 2 complete PRP sessions with minimal prompting
- Write both records fully and rebook both clients
- Deliver the aftercare counselling in Telugu, unaided

Safety & contraindications

- Any fever, spreading redness or pus after PRP = doctor the same day
- Never proceed if the client is unwell, fasting-weak or has had recent fever

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 45–60 minutes per session
- No head wash 24 h
- Next session in 4 weeks
- Aftercare on WhatsApp the same evening

Homework

Write the full PRP session SOP from arrival to rebooking, from memory.

Grooming & professional tip

Clients judge PRP by how organised the room felt, not by the syringe. Smooth, sterile, narrated — that is the whole experience.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 16
16 of 30 · Sundays holiday

GFC therapy — science, preparation & practical

GFC థెరపీ — సైన్స్ & ప్రాక్టికల్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Explain GFC and how it differs from PRP
- Prepare GFC exactly as per kit protocol
- Counsel the client on choosing between PRP and GFC

THEORY · థియరీ

What GFC is

Growth Factor Concentrate is prepared from the client's own blood using a kit that activates platelets and then removes the cells, giving a pure, cell-free concentrate of growth factors at a higher and more consistent concentration than standard PRP. Preparation takes longer (activation + incubation + spin) and uses a branded kit.

PRP vs GFC

PRP: lower cost, faster preparation, well-established, results depend on the client's platelet quality. GFC: higher and standardised growth factor concentration, often less discomfort, usually fewer sessions (3–4), higher cost. Both need maintenance. The doctor chooses based on grade of loss, budget and response — you explain the difference honestly, never push the costlier one.

Preparation discipline

Kit protocols differ by brand: draw volume, activator, incubation time and temperature, spin speed and time. Deviating from any of these wastes the kit and the client's money. Read the insert every time until it is second nature, and keep one printed copy on the trolley.

Hands-on today

- Prepare GFC under supervision, following the kit insert step by step
- Compare PRP and GFC preparation times and outputs in writing
- Explain PRP vs GFC to a partner in 2 minutes

Safety & contraindications

- Incubation time and temperature are not flexible — follow the insert
- Discard any kit whose seal is broken or whose expiry has passed

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- GFC = cell-free growth factors
- Usually 3–4 sessions
- Follow the kit insert exactly
- Doctor decides PRP vs GFC

Homework

Write a side-by-side PRP vs GFC comparison card you could hand a client.

Grooming & professional tip

When a client asks 'edi better?', answer with their grade and goal, not with price. Honest guidance sells the right treatment, and the right treatment gives results.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Hair mesotherapy — cocktails & technique
హెయిర్ మెసోథెరపీ

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand mesotherapy cocktails and their role in a hair plan
- Assist a mesotherapy session correctly
- Combine meso with PRP/GFC in a sensible calendar

THEORY · థియరీ

What goes in

Ready-made ampoules combining vitamins (biotin, B-complex), amino acids, peptides, hyaluronic acid, trace minerals (zinc, copper peptides) and sometimes plant extracts. Delivered superficially into the scalp to improve the follicular environment, circulation and hair shaft quality. It is supportive therapy — it does not replace PRP/GFC or the doctor's medical plan.

Technique

Micro-injections 0.05 ml per point, 1 cm grid, very superficial (2–4 mm), using a 30–32 G needle or a meso gun/dermapen with adjustable depth. Session 20–30 minutes. Typical course: 6–8 sessions, weekly or fortnightly, then monthly maintenance. Mild redness and pinpoint bleeding settle within hours.

Calendar with other treatments

Common pattern: PRP/GFC monthly, meso in the alternate fortnight, scalp treatments between, home care daily. Never do meso and PRP on the same day. Keep 7 days from any chemical service, and pause everything if the scalp is inflamed.

Hands-on today

- Assist one full mesotherapy session and document it
- Practise grid marking and depth setting on a mannequin
- Build a 3-month combined PRP + meso calendar

Safety & contraindications

- Check allergies before any cocktail; stop at the first sign of a reaction
- Single-use needles and cartridges only

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Superficial, 2–4 mm
- 6–8 sessions then maintenance
- Never same day as PRP
- Supportive, not a replacement

Homework

Write the contents and purpose of your clinic's meso ampoule in simple client language.

Grooming & professional tip

Explain meso as 'follicle ki food', PRP as 'follicle ki signal'. Simple images make clients accept combination plans.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 18

18 of 30 · Sundays holiday

Microneedling, derma roller & LLLT for scalp

మైక్రోనీడ్లింగ్, డెర్మా రోలర్ & LLLT

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Use scalp microneedling correctly and safely
- Explain LLLT and home devices honestly
- Advise on home derma roller use without causing harm

THEORY · థియరీ

Scalp microneedling

Controlled micro-injury with a dermapen or roller (0.5–1.5 mm on scalp) stimulates growth factors and wound-healing signals, and improves absorption of topical therapies. Evidence is best when combined with the doctor's topical plan. In clinic: 1.0–1.5 mm, every 3–4 weeks. Immediate pinpoint bleeding and redness are expected.

LLLT

Low-level laser therapy (caps, combs, in-clinic panels) at 650–670 nm is thought to stimulate follicular mitochondria. Results are modest, slow (4–6 months) and only with consistent use 3× weekly. Honest positioning: an add-on for compliance-driven clients, not a standalone solution. Never promise regrowth from a cap alone.

Home devices

If the doctor approves home rolling: 0.5 mm maximum, once a week, on a clean scalp, device sterilised in 70% alcohol before and after, replaced every 8–10 uses, never on active infection or inflamed scalp, and no topical applied immediately after unless advised. Most home damage comes from daily rolling with dirty, blunt needles.

Hands-on today

- Perform supervised scalp microneedling on a live client
- Demonstrate correct home roller technique and sterilisation
- Explain LLLT honestly to a client asking about a laser cap

Safety & contraindications

- No microneedling over active infection, inflamed scalp or scarring alopecia
- Never share a roller between clients — single client, single device

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Clinic 1.0–1.5 mm, every 3–4 weeks
- Home 0.5 mm, weekly, sterile
- LLLT: modest, 4–6 months, 3× weekly
- Combination beats any single tool

Homework

Write the home derma roller do's and don'ts card in Telugu.

Grooming & professional tip

Clients buy gadgets online and damage their scalp. Being the person who teaches safe use keeps them in your clinic instead of on the internet.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 19

19 of 30 · Sundays holiday

Combination protocols & treatment course design

కాంబినేషన్ ప్రోటోకాల్స్ & కోర్సు డిజైన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Design a complete 6-month hair programme
- Sequence procedures safely with correct gaps
- Present the programme so the client commits to it

THEORY · థియరీ

Designing the programme

Month 0: investigations, scalp correction, photos, home care start. Months 1–4: PRP or GFC monthly, meso or microneedling in between, doctor's topicals daily. Months 5–6: reduce to every 6–8 weeks, review photos at month 3 and month 6, adjust. Beyond: maintenance every 2–3 months, permanently for pattern loss.

Gap rules

PRP and meso: not the same day, 2 weeks apart is ideal. Microneedling and PRP: 2 weeks apart. Scalp peel: 3+ days before any injection. Colour/chemical: 7 days either side. Hair spa: 48–72 hours after injections. Write the whole calendar with dates on one page for the client.

Commitment conversation

Show the calendar, the cost per month, and the month-3 and month-6 photo checkpoints. Explain what happens if sessions are skipped — the follicle cycle does not wait. Offer package pricing approved by the doctor and rebook the next two sessions before they leave.

Hands-on today

- Design 3 full programmes: male Norwood II, female post-partum TE, female Ludwig II
- Present one to the trainer as a client-facing plan with dates
- Check each plan against the gap rules

Safety & contraindications

- Every programme needs the doctor's approval before it is offered
- Pause the programme for infection, fever, pregnancy or any new medical issue

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Month 3 and month 6 photo checkpoints
- Never PRP + meso the same day
- Maintenance is permanent in AGA
- Book two sessions ahead

Homework

Draw a 6-month calendar for a male Norwood III client with exact dates.

Grooming & professional tip

Hair programmes fail on attendance, not on science. The clinic that books, reminds and follows up wins on results.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 20

20 of 30 · Sundays holiday

Complications, infection control & emergencies

కాంప్లికేషన్స్ & ఎమర్జెన్సీ

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Prevent the complications specific to hair procedures
- Respond correctly in the first minutes
- Document and escalate professionally

THEORY · థియరీ

What can go wrong

Vasovagal faint during draw or injection; bruising and swelling (especially forehead swelling after frontal injections, appearing next morning); severe headache; folliculitis; infection at injection sites; allergic reaction to numbing cream or meso cocktail; needle-stick injury to staff; and rarely, worsening shedding for 2–4 weeks after the first session.

First response

Faint: flat, legs up, loosen clothing, monitor, call doctor. Swelling: cold compress, reassure, head elevated while sleeping, resolves in 2–3 days. Suspected infection (pain, pus, fever, spreading redness): doctor the same day, photograph, do not start any cream yourself. Allergic reaction (itching, hives, breathlessness): stop, call for help immediately, follow emergency protocol.

Prevention

Full history and allergy check, sterile technique end to end, correct numbing amount, well-fed hydrated client, correct depth, and honest pre-counselling about swelling and temporary shedding. Documentation of every session's details is what lets the doctor decide quickly when something is off.

Hands-on today

- Simulate faint, forehead swelling and suspected infection scenarios
- Locate the emergency kit and rehearse the escalation chain
- Write an incident report for a simulated event

Safety & contraindications

- Never treat a febrile or unwell client
- Needle-stick: wash, report, follow protocol immediately — do not hide it

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Forehead swelling: next morning, 2–3 days
- Temporary shedding can occur — pre-counsel it
- Infection = doctor same day
- Document everything, photograph everything

Homework

Write the emergency escalation chart for your clinic and memorise it.

Grooming & professional tip

Pre-counselling is complication management. A client warned about forehead swelling calls you calmly; one who was not, posts a review.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 21

21 of 30 · Sundays holiday

Case studies & treatment planning workshop

కేస్ స్టడీస్ & ప్లానింగ్ వర్క్‌షాప్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Work through real anonymised cases end to end
- Defend your plan with reasons
- Learn from the trainer's corrections

THEORY · థియరీ

How to read a case

Age and sex → pattern and duration → trigger → medical and family history → previous treatments → scalp findings and trichoscopy → photos → doctor's diagnosis. Then: what must be corrected first, what procedures, what home care, what timeline, what checkpoints.

Common traps

Starting injections before treating a dandruff/seborrhoea flare; ignoring anaemia or thyroid; promising density to a Norwood V; treating a scarring alopecia cosmetically; and not photographing Day 0. Each of these turns a good clinical outcome into an unhappy client.

Presenting to the doctor

Two sentences of history, key findings, your proposed plan, and your question. Doctors value a therapist who presents crisply — it is how you earn responsibility and higher-value work in any clinic.

Hands-on today

- Work through 5 real anonymised case files in pairs
- Present 2 plans to the trainer and take corrections
- Rewrite one plan after feedback

Safety & contraindications

- When findings do not match the plan, stop and ask — never proceed on assumption

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Correct the scalp before injecting
- Rule out anaemia/thyroid/PCOS
- Day-0 photos always
- Present crisply to the doctor

Homework

Write up one case fully as if for the clinic file, including your plan and rationale.

Grooming & professional tip

Keep a personal case book from day one of your job. In two years it becomes the portfolio that gets you a senior role.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 22

22 of 30 · Sundays holiday

Client communication, expectations & retention

క్లయింట్ కమ్యూనికేషన్ & రిటెన్షన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Set honest expectations that keep clients in treatment
- Handle the common hair-client objections
- Run the retention system

THEORY · థియరీ

Expectation script

'Mundu shedding taggutundi — 4 to 6 weeks lo. Kotha hair 3 nelalaki kanipistundi. Density 5–6 nelalaki. Mana goal: loss ni aapadam, taruvata density penchadam. Pattern loss ki maintenance permanent ga kavali.' Say it, write it, and photograph Day 0 in the same visit.

Objections

'PRP cheyinchukunte hair vastunda?' → explain arrest-then-density. 'Chala costly' → break into monthly cost, compare with repeated product spending. 'Nenu oil raasukovacha?' → explain timing. 'Friend ki pani cheyaledu' → explain that grade, cause and compliance differ. 'Transplant better kada?' → explain candidacy honestly and route to consultation.

Retention

Rebook before they leave. WhatsApp aftercare the same evening. 24-hour call after injections. Reminder 3 days before the next session. Photo review at month 3 shown side by side. Win-back message if they miss two months. Our WhatsApp agent runs most of this — you keep the data accurate.

Hands-on today

- Role-play 5 objections, twice each
- Deliver the full expectation script in Telugu unaided
- Practise a month-3 photo review conversation

Safety & contraindications

- Never promise regrowth on scarred or long-lost areas
- Never compare one client's results with another's

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Shedding 4–6 weeks, growth 3 months, density 5–6 months
- Rebook before leaving
- Month-3 photo review
- Maintenance is permanent in AGA

Homework

Record yourself giving the expectation script and listen back twice.

Grooming & professional tip

Show the month-3 comparison on a big screen, not a phone. Clients who can see the difference buy the maintenance plan without hesitation.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 23

23 of 30 · Sundays holiday

Clinic operations, WhatsApp flow & documentation

క్లినిక్ ఆపరేషన్స్ & WhatsApp ఫ్లో

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Run a hair clinic day efficiently
- Use the WhatsApp agent and staff dashboard correctly
- Keep documentation a doctor can rely on

THEORY · థియరీ

The day

Open: check centrifuge, stock of kits/tubes/needles, autoclave load, review today's cases and pull files. During: keep slot discipline (PRP needs 60 minutes, not 30), log as you go. Close: sterilise, segregate waste, restock, reconcile records, confirm tomorrow, report low stock — especially PRP/GFC kits, which cannot be improvised.

WhatsApp agent + dashboard

The AI assistant handles enquiries 24×7, books, reminds, sends aftercare and follow-ups, and creates lead cards with call-prep notes. Staff use dermaluxe.ai/staff.html: hot leads first, call, set status, write a note. For academy students there is also a daily material and tips flow — the same system that is teaching you now.

Documentation

Every session: date, procedure, kit lot number, volume, points, doctor, reactions, aftercare given, next date, photos. Lot numbers matter — if a batch is recalled or a reaction is reported, you must be able to trace it.

Hands-on today

- Run a mock day with 3 PRP cases and 2 consultations
- Update 5 leads on the staff dashboard with call notes
- Do a stock count including kits and tubes, and write the indent

Safety & contraindications

- Never start a PRP list without confirming kit stock and expiry dates
- Never double-book injection slots

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- PRP slot = 60 minutes
- Record lot numbers
- Log as you go
- Weekly stock and expiry check

Homework

Write opening and closing checklists for a hair clinic day.

Grooming & professional tip

Expiry-check the kit box in front of the client. It looks professional and it protects you.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 24

24 of 30 · Sundays holiday

Revision + full live-client day

రివిజన్ + లైవ్ క్లయింట్ డే

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Work a full day as the clinic hair therapist under observation
- Handle every step without prompting
- Find your own remaining gaps before the exam

THEORY · థియరీ

Today's format

You are rostered as the therapist: your clients, your room, your records. The trainer observes and steps in only for safety. Each client must get the full journey — consultation check, consent, analysis, procedure, aftercare, record, rebooking.

Assessment points

Sterile technique, centrifuge and preparation accuracy, client handling and language, documentation, aftercare counselling, time management, and how you ask for help when unsure.

Self-review

After every client, write one strength and one fix. Bring this list to tomorrow's mock exam so your practice is targeted, not random.

Hands-on today

- Minimum 3 complete client journeys (one PRP/GFC, one scalp treatment, one consultation)
- Full records for each, submitted for review
- Written self-review after each client

Safety & contraindications

- Call the trainer before any step you are not sure about — that is a strength, not a weakness

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Full journey every time
- Records same day
- One strength + one fix per client

Homework

List your three weakest areas and the drill for each before tomorrow.

Grooming & professional tip

The habit of self-review after every client is what makes a therapist senior in two years instead of six.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 25
25 of 30 · Sundays holiday

Mock practical, viva prep & portfolio
మాక్ ప్రాక్టికల్, వైవా & పోర్ట్‌ఫోలియో

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Complete a full mock practical under exam conditions
- Answer viva questions with structure
- Assemble your professional portfolio

THEORY · థియరీ

Exam format

Practical (60%): hygiene and set-up, consultation and consent, assigned procedure (PRP preparation and assist, scalp treatment, or analysis and planning), aftercare and documentation. Viva (40%): hair cycle, loss patterns, PRP/GFC science, protocols and complications, plus one case plan. Pass 70%.

Viva structure

What it is → how it works → who it suits → contraindications → protocol numbers → aftercare. If unsure: 'Idi doctor ni confirm chesi cheptanu.' That answer is safe in an exam and correct in a clinic.

Portfolio

Certificate, consented before/after photo sets, session logs, protocols you have written, CV and a 60-second self-introduction. For hair roles, employers especially want to see that you can prepare PRP/GFC correctly and keep records.

Hands-on today

- Full mock practical under exam conditions
- 25-question rapid viva
- Assemble the portfolio folder and CV draft

Safety & contraindications

- Any breach of sterility or consent in the exam is an automatic fail

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Practical 60% + viva 40%, pass 70%
- Structure every answer
- Portfolio: photos + logs + CV
- Honesty beats guessing

Homework

Finish your CV and record your 60-second introduction in Telugu and English.

Grooming & professional tip

Photograph your own prepared PRP syringes through the course (no client data). It is the most convincing page in a hair therapist's portfolio.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 26

26 of 30 · Sundays holiday

Advanced session: combination clinics & upselling ethically

అడ్వాన్స్‌డ్ సెషన్ — కాంబినేషన్ & ఎథికల్ అప్‌సెల్లింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Combine hair and skin services in one client journey
- Upsell ethically — only what the client actually needs
- Build monthly recurring revenue for the clinic

THEORY · థియరీ

Natural combinations

Hair fall client with dandruff → add scalp treatment series. Bride with hair thinning → hair programme + bridal skin package aligned to the wedding date. Post-partum client → TE programme + pigmentation care. PCOS client → hair, acne and unwanted facial hair (LHR) together, coordinated with the doctor.

Ethical upselling

Rule one: recommend only what the findings justify. Rule two: always give the client the option to start with the essential and add later. Rule three: never use fear. A therapist who says 'idi ippudu avasaram ledu' is trusted for years and sells far more over time than one who pushes packages.

Recurring revenue

Maintenance PRP/GFC every 2–3 months, monthly hair spa, quarterly scalp detox, and home-care refills are the backbone of a clinic's steady income. Set reminders for refills — a client who runs out of shampoo often stops the whole routine.

Hands-on today

- Build combined journeys for 3 real profiles
- Practise an ethical upsell conversation and a 'not needed now' conversation
- Design a refill reminder schedule

Safety & contraindications

- Never bundle a medical decision into a package — the doctor decides treatment, not the offer

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Recommend from findings, never fear
- Essential first, add later
- Maintenance + refills = steady revenue
- Coordinate with the doctor

Homework

Write a combined 3-month plan for a PCOS client covering hair, skin and LHR.

Grooming & professional tip

The sentence 'idi meeku ippudu avasaram ledu' is the most profitable thing you will ever say. Trust compounds.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 27

27 of 30 · Sundays holiday

Hair transplant awareness & referral (observation only)

హెయిర్ ట్రాన్స్‌ప్లాంట్ అవేర్‌నెస్ & రెఫరల్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Explain transplant options accurately at a counselling level
- Identify who is and is not a candidate
- Refer correctly without over-promising

THEORY · థియరీ

The basics

Follicles from the permanent (DHT-resistant) donor area at the back and sides are moved to thinning areas. FUE extracts follicles individually; DHI implants with a pen for dense, precise placement. Day procedure under local anaesthesia, new growth from 3–4 months, full result 9–12 months, permanent because the moved follicles keep their donor character.

Candidacy

Good candidates: stable pattern loss with a good donor area, realistic goals, medically fit. Poor candidates: very young with rapidly progressing loss, poor donor density, active scarring alopecia, uncontrolled medical conditions, or unrealistic expectations. Medical therapy must continue after a transplant — otherwise the untransplanted native hair keeps thinning.

Your role

Explain honestly, answer basic questions, show the clinic's own consented results, and book a surgeon consultation. Never quote graft numbers or prices yourself, never promise density, and never disparage a client's previous transplant elsewhere.

Hands-on today

- Observe a consultation or procedure if scheduled; otherwise study case documentation
- Practise a transplant counselling conversation
- List 6 candidacy questions you would ask before referring

Safety & contraindications

- Never advise stopping medical therapy after a transplant
- Any post-transplant pain, pus or fever → surgeon immediately

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- FUE vs DHI
- Growth 3–4 months, full 9–12 months
- Medical therapy continues after transplant
- Therapists counsel, surgeons decide grafts

Homework

Write a one-page transplant FAQ in Telugu for the front desk.

Grooming & professional tip

Clients ask about transplant when they are losing hope in medical care. Answer honestly — the honest answer usually keeps them in the clinic.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 28

28 of 30 · Sundays holiday

Records, photography review & progress reporting

రికార్డులు, ఫోటో రివ్యూ & ప్రోగ్రెస్ రిపోర్టింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Produce a professional 3-month progress report
- Review photographs objectively
- Use the report as a clinical and retention tool

THEORY · థియరీ

What a progress report contains

Client ID, start date, diagnosis, sessions done with dates, home care compliance, Day-0 vs current photos in the same five views, trichoscopy comparison, pull test change, client's own rating, therapist's observation, doctor's note, and the plan for the next quarter.

Reviewing photos honestly

Same views, same light, same parting, same camera distance. Look at density at the parting, vertex coverage, hairline stability and miniaturisation. If there is no change, say so and investigate why — compliance, diagnosis, deficiency, or the need to escalate. Manipulating angles or lighting destroys trust permanently.

Using the report

Deliver the report in a scheduled 15-minute review appointment, not in the corridor. Clients who see a structured report continue treatment; clients who get a casual 'baguntundi' quietly drift away.

Hands-on today

- Prepare two full progress reports from real files
- Conduct a mock review appointment with the trainer as client
- Audit 5 photo sets for view and lighting consistency

Safety & contraindications

- Never edit or filter clinical photographs
- Photo use for marketing requires separate written consent every time

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Same 5 views every visit
- Month-3 structured review
- No change = investigate, do not hide
- Never filter clinical photos

Homework

Build your own progress report template and fill it for one case.

Grooming & professional tip

Book the month-3 review at the first visit. An appointment in the diary is what makes the review actually happen.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 29

29 of 30 · Sundays holiday

Final revision & exam readiness

ఫైనల్ రివిజన్ & ఎగ్జామ్ ప్రిపరేషన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Close every remaining knowledge and skill gap
- Rehearse the complete practical flow twice
- Be exam-ready and clinic-ready

THEORY · థియరీ

Revision map

Hair cycle and loss patterns → scalp conditions → analysis and documentation → scalp treatments → PRP science, preparation and assist → GFC → meso and microneedling → combination planning → complications → communication and operations. Rate yourself 1–5 on each and drill the lowest three.

The numbers to know cold

50–100 hairs/day normal; 85–90% anagen; pull test >6 hairs positive; PRP 4–6 ml, 1 cm grid, 4–6 mm depth, monthly ×4; GFC 3–4 sessions; meso 6–8 sessions; microneedling 1.0–1.5 mm clinic / 0.5 mm home; shedding down 4–6 weeks, growth 3 months, density 5–6 months; no head wash 24 h.

Exam-day discipline

Arrive early, uniform ready, nails and hair in order, know where everything is in the room, read the case sheet fully before you start, verbalise your safety checks, and never skip consent.

Hands-on today

- Two full practical rehearsals under time pressure
- Rapid-fire numbers quiz, 30 questions
- Final portfolio check with the trainer

Safety & contraindications

- If you are unsure in the exam, say what you would do and who you would ask — that is a correct clinical answer

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Know the numbers cold
- Verbalise safety checks
- Consent before everything
- Drill your three weakest areas

Homework

Self-rate all 10 revision areas and drill the lowest three tonight.

Grooming & professional tip

Confidence on exam day comes from rehearsal, not revision. Do the flow with your hands twice more than you read it.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 30

30 of 30 · Sundays holiday

Final practical exam, viva & career guidance

ఫైనల్ ప్రాక్టికల్, వైవా & కెరీర్ గైడెన్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Pass the final practical and viva
- Receive your certificate and portfolio feedback
- Plan your next 6 months in the hair-care industry

THEORY · థియరీ

Today

Morning: practical exam — full client journey with an assigned procedure, scored live. Afternoon: viva. Evening: results, certificate presentation, group photo and placement discussion for eligible trainees.

Career paths

Hair therapist or trichology assistant in a doctor-led clinic (our clinics hire from each batch first), PRP/GFC specialist therapist, clinic manager, brand trainer for hair-care companies, or transplant team assistant with further training. Skill in preparation and documentation is what gets you promoted.

Next 6 months

One new skill a month, a personal case book, consented photo sets, a settings and protocol diary, and continued learning through the alumni group. Refresher sessions and new-protocol updates are open to all DermaLuxe Academy alumni.

Hands-on today

- Final practical exam (scored)
- Final viva (scored)
- Certificate presentation and portfolio review

Safety & contraindications

- Carry the sterility and documentation discipline you learned here into every clinic — hair procedures involve blood, and shortcuts are never acceptable

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Pass mark 70%
- Certificate + internship letter (2-month course)
- Alumni group for updates and jobs
- Priority hiring at our locations

Homework

Write your 6-month goal: role, skill, income and one measurable target.

Grooming & professional tip

You now handle people's blood and their confidence. Stay sterile, stay honest, keep asking questions — that is a career, not a job.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్